INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Osimertinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern

Ongoing

Follow the prescribed daily plan

Regimen source

NCCP 00353

Version 5

Daily oral treatment without a rest week

Treatment stages

01 **Daily treatment**

02 **Reviews**

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Sources and further information	15

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Osimertinib blocks abnormal EGFR growth signals in selected lung cancers. After complete surgery, treatment aims to lower the risk of cancer returning. In advanced cancer, it aims to control cancer and delay progression. Confirm which aim applies to you.

EGFR TARGETED THERAPY

Osimertinib

Blocks abnormal EGFR growth signals

EGFR mutation identified → Growth signal blocked → Cancer growth slowed

Osimertinib is an EGFR tyrosine kinase inhibitor. It blocks signalling from sensitising EGFR mutations and the T790M resistance mutation, reducing cancer-cell growth signals. The tumour test and treatment setting determine whether it is appropriate.

Report new breathlessness, severe rash, diarrhoea, fainting, unusual bleeding or eye pain promptly.

Your treatment aim

Which setting?

This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

NCCP 00353 v5 · official regimen · EMA Tagrisso product information (updated 18/08/2026)

Duration

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

NCCP 00353 v5 · official regimen

What studies tell us

The evidence differs by setting. In a study after complete surgery, adjuvant osimertinib improved survival compared with placebo. In untreated advanced EGFR-mutated cancer, it improved survival compared with older EGFR tablets. After resistance with T790M, it delayed progression compared with platinum/pemetrexed, but the final overall-survival comparison was not statistically significant. These results describe selected study groups and cannot predict your own outcome.

Your own decision

Ask how closely the study population matches your situation and discuss alternatives, harms and remaining uncertainty.

ADAURA final overall survival, Tsuboi 2023 · FLAURA final overall survival, Ramalingam 2020 · AURA3 primary progression-free survival, Mok 2017 · AURA3 final overall survival, Papadimitrakopoulou 2020

Evidence depends on the treatment setting

Adjuvant and advanced trials answer different questions.

Study averages cannot give an individual prognosis or compare today's treatment options head to head. Chemotherapy-combination and post-chemoradiation studies require a separate discussion.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team urgently for new or worsening breathlessness, cough or fever; fainting, palpitations or chest pain; blistering/peeling skin or mouth/eye/genital sores; unusual bruising, bleeding or marked pallor; severe diarrhoea, dehydration or inability to drink. New eye pain, light sensitivity or blurred vision needs prompt advice. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Relative stages, not equal periods or booked appointments. Tablets are continuous; cycle 2 is a source review label. Your team confirms who arranges and reviews each test.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00353 v5 · official regimen · EMA Tagrisso product information (updated 18/08/2026)

THE WHOLE TREATMENT JOURNEY

Osimertinib monotherapy: treatment and checks

Treatment stages

01 **Daily treatment**

02 **Reviews**

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EGFR-mutated non-small-cell lung cancer (NSCLC) · adjuvant or advanced

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Early review / cycle 2	Ongoing daily treatment	Benefit / toxicity review	Completion or change
Treatment NCCP 00353 · SMPC	Confirm adjuvant or advanced plan	Daily osimertinib; no planned rest week	Continue prescribed daily dose	Review benefit and harms	Adjuvant three-year review; advanced plan individualised
Blood tests NCCP 00353 · SMPC	Blood count, liver, kidney and bone profile	Repeat at agreed reviews	Continue regular tests	Extra tests for symptoms or abnormalities	Follow unresolved changes
Heart monitoring NCCP 00353 · SMPC	electrocardiogram (ECG); left ventricular ejection fraction (heart pumping measurement) (LVEF) if indicated	ECG at NCCP cycle-2 review	ECG from cycle 3 if indicated	LVEF and electrolytes as indicated	Confirm recovery before any restart

Care / check	Before the first dose	Early review / cycle 2	Ongoing daily treatment	Benefit / toxicity review	Completion or change
Symptoms and medicines NCCP 00353 · SMPC	Review lungs, skin, bowel, eyes and interactions	Report symptoms promptly	Check adherence and tablet supply	Review toxicities and every medicine	Continue symptom advice and reproductive precautions
Cancer assessment NCCP 00353 · SMPC	Confirm mutation, stage and treatment aim	Review clinical progress	Scans according to Consultant plan	Discuss response, recurrence or progression	Agree follow-up or next treatment

What each check is for

1 **Confirm the indication**

Before the first dose

Adjuvant and advanced evidence and duration differ.

Ask / act: Document the exact NCCP a/b/c indication and intended duration.

2

Early heart review

Baseline and NCCP cycle 2

QT prolongation can be clinically silent.

Ask / act: Book the early review without introducing a tablet rest week.

3 **Possible serious toxicity**

At any time

Lung, heart, skin or marrow effects may require interruption or permanent stopping.

Ask / act: Urgent assessment and an authorised treatment plan.

4 **Adjuvant completion / advanced continuation**

At the planned duration and benefit reviews

Advanced indefinite duration must not be copied into adjuvant treatment.

Ask / act: Agree completion, follow-up or the next treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Tests and symptoms guide safe treatment decisions.

Before starting

Confirm the relevant EGFR test, treatment setting and prescription. Adjuvant treatment needs complete resection and the specific stage/mutation criteria. Blood count, kidney, liver and bone profile; ECG before treatment. Consider an echocardiogram or multigated acquisition (heart pumping) scan (MUGA) LVEF assessment for cardiac risks or symptoms; review all medicines, pregnancy, contraception and breastfeeding.

During treatment

Repeat blood count, kidney, liver and bone profile at the agreed reviews; extra checks may be needed for symptoms or abnormal results. NCCP specifies ECG at baseline and cycle 2, then cycle 3 onwards if clinically indicated. The team translates cycle 2 into a booked review; the tablets remain daily, without a treatment break. LVEF assessment if clinically indicated; ECG and electrolytes with cardiac/QT risks. Scans and disease checks follow the Consultant plan.

Taking osimertinib

Follow your own prescription.

Daily treatment

Take at the same time each day, with or without food. Swallow whole with water; do not crush, split or chew. If you forget a dose, take it when remembered unless less than 12 hours remain before the next dose, when it should be skipped. Confirm the instruction with your pharmacist if the next dose is exactly 12 hours away. Do not take an extra dose without advice if you vomit. Ask for the approved water-dispersion instructions if swallowing is difficult.

Interactions

Tell the team about all medicines and supplements. Do not take St John’s wort. Strong enzyme-inducing medicines and medicines that affect heart rhythm require pharmacy review before changes.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily tablets, reviews and treatment decisions

No dates or personal information are needed. Confirm your own appointments with the team.

Treatment or check	Timing in this view
Preparation	**Before the first dose** Confirm indication, mutation, prescription, blood tests and electrocardiogram (ECG).
Daily oral treatment	**Early review / cycle 2 → Ongoing daily treatment → Benefit / toxicity review** The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.
Tests and symptoms	**Early review / cycle 2 → Ongoing daily treatment → Benefit / toxicity review → Completion or change** Keep blood and heart reviews; report important symptoms between visits.
Duration and next plan	**Completion or change** Adjuvant therapy has a three-year studied duration; advanced therapy has no fixed course cap.

Take at the same time each day, with or without food. Swallow whole with water; do not crush, split or chew. If you forget a dose, take it when remembered unless less than 12 hours remain before the next dose, when it should be skipped. Confirm the instruction with your pharmacist if the next dose is exactly 12 hours away. Do not take an extra dose without advice if you vomit. Ask for the approved water-dispersion instructions if swallowing is difficult.

Columns show sequence, not equal lengths of time. Do not restart clinically held treatment from this educational timeline.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Living with treatment

Report symptoms early and keep your contact numbers available.

Skin and nails

Agree an EGFR-inhibitor skin-care plan from the start and report rash or painful nail changes early. Blistering, peeling or mucosal sores need urgent assessment.

Bowel symptoms and hydration

Report diarrhoea early, follow the prescribed symptom plan and maintain fluids if able. Severe diarrhoea, weakness or reduced urine needs urgent advice.

EMA Tagrisso product information (updated 18/08/2026)



Nausea medicines

Osimertinib has minimal-to-low oral emetic risk. Routine infusion premedication is not usually needed. Agree rescue medicines individually, including QT and interaction review; do not copy a chemotherapy antiemetic course.

NCCP 00353 v5 · official regimen · EMA Tagrisso product information (updated 18/08/2026) · NCCP NCIS antiemetics V8 (2025): parenteral scope; oral support individualised



Medicine interactions

Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment. Effective contraception continues for at least two months after the final dose for women and four months for men. Osimertinib may reduce hormonal contraceptive effectiveness; discuss a suitable method. Do not breastfeed during treatment. Discuss fertility and plans for pregnancy before starting.

When to get urgent help

Do not wait until the next appointment.

Call urgently

Contact the oncology team urgently for new or worsening breathlessness, cough or fever; fainting, palpitations or chest pain; blistering/peeling skin or mouth/eye/genital sores; unusual bruising, bleeding or marked pallor; severe diarrhoea, dehydration or inability to drink. New eye pain, light sensitivity or blurred vision needs prompt advice. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When you call

Give your medicine name, last dose and symptoms. Explain whether you can drink, pass urine and manage usual activities. Do not delay a call while collecting information.

EMA Tagrisso product information (updated 18/08/2026)

When treatment changes

Your team authorises continuation, interruption, restart or completion.

Adjuvant review

Confirm the planned completion point at up to three years and your follow-up plan.

NCCP 00353 v5 · official regimen

Advanced disease

Continue only while benefit and tolerability remain acceptable; a new symptom or progression may change the plan.

NCCP 00353 v5 · official regimen

Practical support

Skin and nails

Bowel symptoms and hydration

Nausea medicines

Medicine interactions

Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Supplies and visits

Arrange repeat prescriptions before tablets run out.

The oral regimen does not itself need an infusion line or pump.

Questions to ask

Do I need dates to use the timeline?

No. It shows relative steps; your team arranges real appointments and reviews results.

Does a rash show treatment is working?

Side effects do not reliably show benefit. Report them; scans and clinical reviews assess cancer control.

What should I ask at consent?

Discuss adjuvant versus advanced intent, expected benefit and uncertainty, alternatives including not proceeding, common skin/bowel effects, serious lung/heart/skin/blood problems, fertility and pregnancy, daily tablets, monitoring and your right to reconsider.

Explain the plan back: Can you explain your treatment aim and duration, how you take the tablet, which heart/blood tests are next, and which symptoms mean you should call today?

Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Osimertinib alone · daily oral treatment

Active medicines: osimertinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Agree an EGFR-inhibitor skin-care plan from the start and report rash or painful nail changes early. Blistering, peeling or mucosal sores need urgent assessment.
- Report diarrhoea early, follow the prescribed symptom plan and maintain fluids if able. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Osimertinib has minimal-to-low oral emetic risk. Routine infusion premedication is not usually needed. Agree rescue medicines individually, including QT and interaction review; do not copy a chemotherapy antiemetic course.
- Tell the team about all medicines and supplements. Do not take St John’s wort. Strong enzyme-inducing medicines and medicines that affect heart rhythm require pharmacy review before changes.
- Avoid pregnancy during treatment. Effective contraception continues for at least two months after the final dose for women and four months for men. Osimertinib may reduce hormonal contraceptive effectiveness; discuss a suitable method. Do not breastfeed during treatment. Discuss fertility and plans for pregnancy before starting.
- NCCP V6 Table 5 supports as-required oral antiemetics for minimal-low risk; select the agent, dose and duration individually after QT, renal and interaction review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eye pain or blurred vision

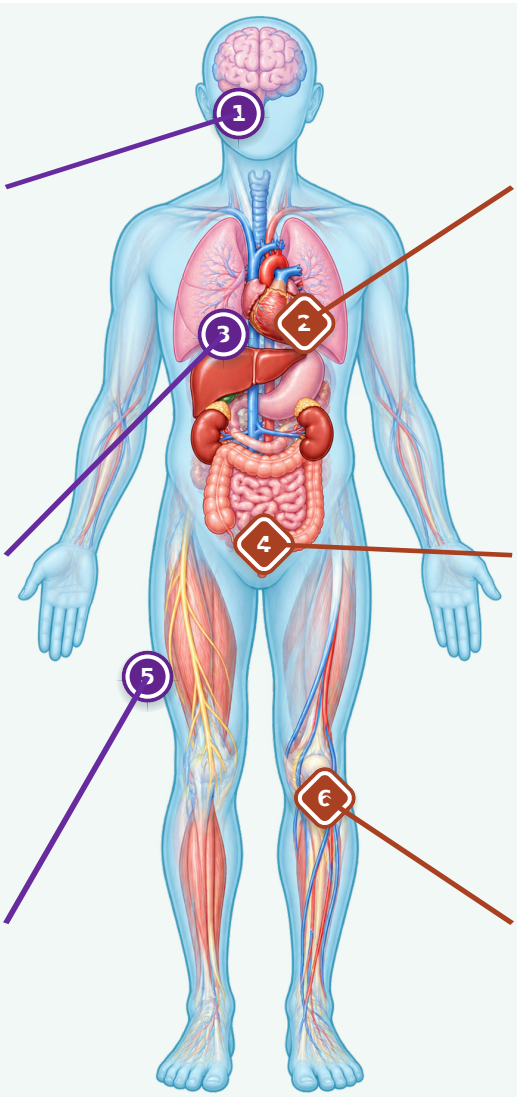
osimertinib

Possible keratitis needs prompt specialist advice.

2 New cough or breathlessness

osimertinib

Possible lung inflammation needs urgent assessment.



3 Palpitations or fainting

osimertinib

Heart rhythm or pumping changes need assessment.

4 Diarrhoea

osimertinib

Report early; severe symptoms or dehydration need urgent advice.

5 Skin and nail changes

osimertinib

Blistering, peeling or mucosal sores are urgent.

6 Bruising or bleeding

osimertinib

Blood-count changes require tests and clinical review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for new or worsening breathlessness, cough or fever; fainting, palpitations or chest pain; blistering/peeling skin or mouth/eye/genital sores; unusual bruising, bleeding or marked pallor; severe diarrhoea, dehydration or inability to drink. New eye pain, light sensitivity or blurred vision needs prompt advice. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00353 v5 · official regimen**
   https://healthservice.hse.ie/documents/6914/353_V5_Osimertinib_Monotherapy.pdf
2. **EMA Tagrisso product information (updated 18/08/2026)**
   https://www.ema.europa.eu/en/documents/product-information/tagrisso-epar-product-information_en.pdf
3. **NCCP NCIS antiemetics V8 (2025): parenteral scope; oral support individualised**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
4. **ADAURA final overall survival, Tsuboi 2023**
   https://www.nejm.org/doi/full/10.1056/NEJMoa2304594
5. **FLAURA final overall survival, Ramalingam 2020**
   https://www.nejm.org/doi/full/10.1056/NEJMoa1913662
6. **AURA3 primary progression-free survival, Mok 2017**
   https://www.nejm.org/doi/full/10.1056/NEJMoa1612674
7. **AURA3 final overall survival, Papadimitrakopoulou 2020**
   https://www.annalsofoncology.org/article/S0923-7534(20)42155-6/fulltext
8. **NCCP V6 Table 5: minimal-low oral therapy uses as-required antiemetics; NCIS V8 is parenteral and does not establish a mandatory oral prescription**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf